NCCP 00446 · clinician draft

Modified FOLFOX-6 + bevacizumab

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Modified FOLFOX-6 + bevacizumab · active treatment cycle

Active medicines: Bevacizumab + Oxaliplatin + Folinic acid + Fluorouracil

Emetogenic risk: Moderate

Medicine	Dose / route	When
Ondansetron	16 mg orally once	Before Day 1 chemotherapy Review QT risk, interactions, constipation and hepatic function.
Dexamethasone	8 mg orally once	Before Day 1 chemotherapy Reconcile all steroid prescriptions; do not duplicate.
Dexamethasone	8 mg orally once daily	Days 2 and 3 after Day 1 chemotherapy Confirm the prescribed delayed prophylaxis and re-anchor it if treatment is delayed.
Metoclopramide	10 mg orally up to three times daily as required	Rescue only; duration to be completed by prescriber Review renal function, neurological contraindications and interactions.

Additional phase-specific support

- Take the prescribed prevention medicines at the planned times and keep rescue treatment available. Tell the team if nausea or constipation is not controlled; do not duplicate steroid or anti-sickness medicines.
- Report diarrhoea promptly and follow the written rescue plan. Severe symptoms, fever, dehydration or inability to drink need urgent advice.
- White-cell growth-factor injections are prescribed according to individual risk and previous complications. They are not automatically required for every patient or every cycle.
- Avoid cold exposure as advised after oxaliplatin. Report persistent numbness or difficulty with buttons, walking or everyday tasks before the next dose.
- Keep the agreed blood-pressure and urine checks. Report bleeding, poorly healing wounds, new swelling or severe headache promptly.
- No routine bevacizumab hypersensitivity premedication unless previous reaction or a specific local plan. Review blood pressure, proteinuria, bleeding and wound healing separately from nausea prevention.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00446 v7b · NCCP NCIS antiemetics V8, Table 1; apply oral regimen guidance separately

Sources checked 2026-10-25 · SACTCheck v0.84.21 · Regimen-level drafts; prescriber and oncology-pharmacy verification required